



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : عايده احمد عبد الخالق مصلحي

العنوان : 0

المحافظة : القليوبية

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 26203111400748 رقم التليفون : 01555355696

العمر (تاريخ الميلاد) : 11-03-1962 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: عايده احمد عبد الخالق مصلح National ID: 26203111400748	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

عائده
احمد عبد الخالق مصلحي
عزبه عزيز عزت القبليه
قليوب - القليوبية

٢٦٢٠٣١١١٤٠٠٧٤٨

JF2315481

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٢٠٢١/١٢ ٢٦٢٠٣١١٤٠٠٧٤٨

ربة منزل
أنثى
مسلمة
ارملة

البطاقة سارية حتى ٢٠٢٨/١٢/٢٩

١١



Name: Aida Ahmed Abdelkhalik

Date: 6/8/2026

BILATERAL BREAST ULTRASOUND

- Increased skin thickness of right breast mainly at the outer quadrants.
- Normal appearance of skin and subcutaneous tissue on the left side.
- A large hypoechoic irregular mass is seen in the right breast at 8 o'clock about 10 cm from nipple measuring 4.5 x 2.5 cm with speculated borders showing internal vascularity.
- Another adjacent hypoechoic irregular mass is seen in the right breast at 9 o'clock about 5 cm from nipple measuring 2.5 x 1.7 cm as well as similar mass at 10 o'clock about 5 CMFN measuring 2.5 x 1.8 cm.
- A complex cystic/solid mass lesion is noted at the left breast at 1 o'clock 3 CMFN with its solid component showing internal vascularity measuring 3.3 x 2.1 cm.
- Left few scattered well-defined lobulated soft tissue mass lesions, the largest is seen at 1 o'clock 5 CMFN measuring 1.4 x 0.9 cm likely fibroadenomas.
- Normal ultrasound appearance of nipples and areolas.
- Enlarged right axillary suspicious LNs are seen with markedly thickened cortex and very small peripherally displaced hilum.
- No suspicious left axillary lymphadenopathy

IMPRESSION:-

- Right breast masses as described highly suggestive of multicentric malignant disease with suspicious right axillary LNs (BIRADS 5).
- Left breast complex mass lesion as described (BIRADS 4) for histopathology.
- Few left breast fibroadenomas as described. (BIRADS 3)

MUCH OBLIGED

Dr. Mohamed Khedr, MSc

Ass.Lecturer of Radiology Ainshams University

BIRADS 1	Normal breast
BIRADS 2	Mostly Benign lesion
BIRADS 3	Probably benign lesion for follow up
BIRADS 4	Suspicious lesion for malignancy (biopsy recommended)
BIRADS 5	Highly suspicious lesion for malignancy
BIRADS 6	Pathologically proven malignant lesion





Name: Aida Ahmed Abdelkhalik

Date: 6 August 2026

MAMMOGRAPHY OF BOTH BREAST

Findings:

- Overall breast composition: the breasts are heterogeneously dense, which may obscure small masses (ACR category c).
- Right breast increased skin thickness.
- Right breast upper and lower outer quadrants suspicious irregular lesions are noted.
- Left breast retroareolar lobulated mass lesion for US correlation.
- No suspicious clusters of microcalcifications are noted.

Impression:

- Right breast suspicious mass for US and histopathology.
- Left breast retroareolar mass lesion as described for US correlation.

MUCH OBLIGED

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BI-RADS classification according to the American College of Radiology

Category	Management	Likelihood of cancer
0 Incomplete	Need further evaluation	Not applicable
1 Negative	Routine screening	0%
2 Benign	Routine screening	0%
3 Probably benign	Short interval follow up	< 2%
4 Suspicious abnormality	Tissue diagnosis	> 2- 95%
5 Highly suggestive of malignancy	Tissue diagnosis	> 95%
6 Biopsy proven malignancy	Surgical excision when clinically appropriate	



Patient Name	عليه احمد عبد الخالق	Case No.	358252
Sex/Age	Female 64 Y	Patient No.	25926
Registered Date	15/8/2026 04:49:01 PM	Referred By Dr.	Interventional Radiology
Validation Date	14/9/2026 12:21:28 PM	Branch	Gustave Roussy
Reporting Date	14/9/2026 01:14:21 PM	Pathology No.	2585/26

Pathology Report

Clinical Data: Right breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 15/08/2026, Cold ischemia time is 20 seconds.

Gross Examination:

-Two grey, white tissue cores, measured 1.2 cm & 1.5 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive breast carcinoma, NST.
- Histologic grade: 1 (Differentiation 2, pleomorphism 1, mitosis 1).
- Ductal carcinoma in situ: Absent.
- Angiolinvasion: Absent.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 70% of tumor cells (score 7/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 30%.

Conclusion:



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة 5 - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

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Right breast mass, Tru-cut biopsy:

- Invasive breast carcinoma (NST), Grade 1.

IPt:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Low (score 1+).
- Ki67%: 30%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature:**Prof. Dr. Rana El Saeed.**

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - معسر القنينة - القاهرة 3 - معسر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadima - Cairo, Egypt

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Signature:

Prof. Dr. Rana El Saeed.



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